

Date: June 9, 2023

From: Office of Occupational Safety and Health (HEFP/19HEFB)

Subject: Clarifications for Community Living Center (CLC) Life Safety Surveys

1. Fire Drills at unexpected times. (KTag-712)

Fire drills in VHA facilities are not required to be conducted “at unexpected times” as identified in K712.

K712 – Fire Drills

Fire drills include the transmission of a fire alarm signal and simulation of emergency fire conditions. Fire drills are held **at unexpected times** under varying conditions, at least quarterly on each shift. The staff is familiar with procedures and is aware that drills are part of established routine. Responsibility for planning and conducting drills is assigned only to competent persons who are qualified to exercise leadership. Where drills are conducted between 9:00 PM and 6:00 AM, a coded announcement may be used instead of audible alarms.

18.7.1.4 through 18.7.1.7, 19.7.1.4 through 19.7.1.7

Issue 23 (copied below) in the VHA-Joint Commission Clarification Document with an effective date of September 4, 2017, documents that VHA does not follow the “at unexpected times” requirement that CMS enforces. Community Living Center surveyors are to follow the guidelines in the VHA-Joint Commission Clarification Document.

Issue 23. Quarterly Fire Drills not required at unexpected times.

While the CMS Life Safety Checklist (Form CMS-2786R (10/2016)) requires quarterly fire drills to be conducted “at unexpected times” (reference Checklist Item K712), NFPA 101, 19.7.1.6 does not require fire drills to be conducted “at unexpected times.” Since VHA facilities are not deemed by TJC, VHA facilities are not required to conduct fire drills at unexpected times and VHA facilities are permitted to conduct fire drills on the same shift, during the same day, and during the same hour, as previous and subsequent quarterly fire drills. See HITF Interpretation June 2017.

2. Fire Drills that are within a building where the CLC is located. (KTag-712)

NFPA 101, 19.7.1.6 requires one drill per shift per quarter in healthcare occupancies. **If the facility performs a fire drill in a building that includes a CLC, that fire drill would count as a fire drill for the CLC provided that CLC staff responded in accordance with the facility fire response plan as required by NFPA 101, 19.7.1.2.** In the case where a fire drill assumes the fire origin to be in a location outside of the CLC portion of the building, the response plan might only require the staff in the CLC to listen to the alarm message/signal to determine the location of the fire origin, but take no further action. In this case, the CLC staff would still be counted as participating in the fire drill. NFPA 101 does not mandate that the fire origin for a fire drill occur in the CLC portion of a building for the drill to count for the CLC.

3. Fire Drills – Transmission of Fire Alarm Signal. (KTag-712)

The term “transmission of a fire alarm signal” as mentioned in NFPA 101, 19.7.1.4 could be interpreted to mean (1) the electronic transmission of an alarm signal to a remote station or fire department, or (2) activation of the notification appliances to notify occupants in the facility. **A fire alarm signal is NOT required to be transmitted to a remote station or fire department when conducting a fire drill. In addition, between 9 p.m. and 6 a.m., a coded announcement may be used in lieu of activating notification appliances for fire drills.** In a healthcare occupancy, when fire drills are conducted, the fire alarm system notification appliances are to be used to provide notification **except** during the hours between 9:00 p.m. and 6:00 a.m., during which time an alternative announcement may be used in lieu of the notification appliances in accordance with NFPA 101 (See background material below). The HITF Interpretation of June 2014 (See [Attachment B](#)) addresses alarm signals during fire drills and a fire alarm signal is NOT required to be transmitted to a remote station or fire department when conducting a fire drill.

===== Background Material =====

NFPA 101(2018), 19.7.1.4 states the following:

19.7.1.4* Fire drills in health care occupancies shall include the transmission of a fire alarm signal and simulation of emergency fire conditions.

However, NFPA 101, 19.7.1.7 provides an exception to 19.7.1.4 as follows:

19.7.1.7 When drills are conducted between 9:00 p.m. and 6:00 a.m. (2100 hours and 0600 hours), a coded announcement shall be permitted to be used instead of audible alarms.

4. Sprinklers in closets and casework (wardrobes/lockers). (KTag-351)

While sprinklers are required in closets within nursing homes, most VHA CLCs are equipped with casework (wardrobes/lockers) for the storage of patient's personal belongings. Casework is different than closets.

Sprinklers are not required in casework (wardrobes/lockers). See [Attachment A](#) for details.

K351 – EXISTING – Sprinkler System – Installation	Nursing homes, and hospitals where required by construction type, are protected throughout by an approved automatic sprinkler system in accordance with NFPA 13, Standard for the Installation of Sprinkler Systems. In Type I and II construction, alternative protection measures are permitted to be substituted for sprinkler protection in specific areas where State or local regulations prohibit sprinklers. In hospitals, sprinklers are not required in clothes closets of patient sleeping rooms where the area of the closet does not exceed 6 ft ² and sprinkler coverage covers the closet footprint as required by NFPA 13, Standard for Installation of Sprinkler Systems. [Nursing homes/skilled nursing facilities require sprinklers in closets, regardless of size, to be fully protected]. 19.3.5.1, 19.3.5.2, 19.3.5.3, 19.3.5.4, 19.3.5.5, 19.4.2, 19.3.5.10, 9.7, 9.7.1.1(1)
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5. Exit discharge characteristics. (KTag-271)

In accordance with NFPA 101, 7.1.6.1.2, VHA permits existing exit discharges that are nonpaved surfaces that are nominally level in accordance with NFPA 101, 7.1.6.3 and that are maintained in accordance with NFPA 101, 7.1.10; and VHA does not require the existing nonpaved surfaces to meet the requirements of NFPA 101, 7.1.6.2. VHA does not follow the CMS requirements that go above the requirements in NFPA 101. The HITF Interpretation of June 2009 (See [Attachment C](#)) addresses “Exit Discharge” and does not mandate a hard packed all weather travel surface from the exit discharge to a public way. Grass, dirt, or gravel may be acceptable in some situations. For example, a grassy surface that leads away from the building in a climate that gets little or no snow would be acceptable in accordance with the HITF interpretation. Remember, as a general rule, exit discharges are not designed for nursing home patients, but for the general public.

K271 – Discharge from Exits	Exit discharge is arranged in accordance with 7.7, provides a level walking surface meeting the provisions of 7.1.7 with respect to changes in elevation and shall be maintained free of obstructions. Additionally, the exit discharge shall be a hard packed all- weather travel surface in accordance with CMS Survey and Certification Letter 05-38. 18.2.7, 19.2.7, S&C 05-38
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In addition, NFPA 7.1.6.1 allows approved existing walking surfaces to be permitted to be continued in use as follows:

7.1.6.1.2 Approved existing walking surfaces shall be permitted.

6. Cooking Facilities. (KTag-324)

This is to clarify that therapy cooking facilities are not required to be protected. There are basically four options in NFPA 101 for providing protection for cooking facilities in healthcare occupancies.

1. No protection. No protection is required for limited cooking or food warming and this includes therapy cooking facilities in health care occupancies. (See NFPA 96, A.1.1.4) Therapy cooking facilities are not required to be protected in accordance with NFPA 96 nor are they required to be protected in accordance with NFPA 101, 18/19.3.2.5.3 or 18/19.3.2.5.4.
2. Full protection in accordance with NFPA 96. This is required for commercial cooking and has no limit on the amount of cooking done or on the number of people served.
3. Protection in accordance with the provisions of NFPA 101, 18/19.3.2.5.3. This applies to residential or commercial cooking facilities that are open to the corridor and used on a routine basis to cook for no more than 30 persons within a smoke compartment. This requires numerous protection provisions as well as smoke detection in accordance with NFPA 101.
4. Protection in accordance with the provisions of NFPA 101, 18/19.3.2.5.4. This is the same as in number 3 above except that the cooking facilities are not open to the corridor and no smoke detection is required for the cooking facilities.

Attachment A
Clarifications for Community Living Center (CLC) Life Safety Surveys: (Rev: 6-9-23)

**Clarification on Sprinklers in Clothes Closets, Wardrobes, Lockers, and
Casework in VA Community Living Centers (Nursing Homes)**
March 29, 2018

The question has come up as to whether sprinklers are required within clothes closets in VA Community Living Centers¹ (CLCs). A discussion of closets should also include wardrobes and lockers since they serve a similar purpose, and in terms of fire protection there is effectively no difference between a closet, a wardrobe, or a locker of the same size and construction.

In the VHA-Joint Commission Clarification Document dated September 4, 2017, Issue 15 (see [Item A in the Appendix](#)) addresses sprinklers in small closets other than clothes closets and wardrobes. For the small closets addressed in Item 15 (such as hose cabinets, hose closets, and electrical closets that are shallow in depth), The Joint Commission will permit sprinklers to be omitted from these spaces provided that the space is small enough so that a person cannot reasonably enter the space. With respect to wardrobe units, Issue 15 refers to NFPA 13 (2016 edition), section 8.1.1(7), and the Healthcare Interpretations Task Force (HITF) Interpretation No. 2 of December 2008. With respect to clothes closets, Issue 15 refers to NFPA 101, 18/19.3.5.10.

NFPA 13 (2016 edition), section 8.1.1(7) permits sprinklers to be omitted from “furniture, such as portable wardrobe units, cabinets, trophy cases, and similar features not intended for occupancy” (see [Item B in the Appendix](#)).

HITF Interpretation No. 2 of December 2008 states that sprinklers are not required to be installed within wardrobes that are attached or freestanding within a fully sheathed alcove, provided that the sprinkler protection of the room includes the floor space of the alcove (see [Item C in the Appendix](#)).

VA considers wardrobes to fall under the category of casework. The VA definition of casework is: “cabinetry and other built-in equipment that is not intended to be portable or moveable. This also includes patient wardrobes which are designed with plumbing and or electrical fixtures built into them.” (Reference: IL 10A4-84-8, February 29, 1984. This document is available for review on the CEOSH web site).

Beginning in 1993, VA eliminated all fire protection requirements for new installations of casework in any sprinkler-protected space. Further guidance on the definition of casework was also provided:

“casework is the built-in cabinetry, built-in wardrobe lockers and other equipment that is not intended to be moveable, but is not furniture.” (Reference: Engineering Management Service National Conference Call Minutes, December 8, 1993, page 1. This document is available for review on the CEOSH web site).

NFPA 101 (2018 edition), sections 18/19.3.5.10 permit sprinklers to be omitted in clothes closets of patient sleeping rooms in hospitals under certain conditions, but this permission is limited to hospitals and does not apply to nursing homes (see [Item B in the Appendix](#)).

In order to apply the above requirements, a distinction must be made between casework (such as a wardrobe or locker) and a closet. In the context of NFPA 13 and NFPA 101, a closet will generally have the same floor level and ceiling height as the room or space into which the closet opens. All of the

¹ VA Community Living Centers were previously known as VA Nursing Home Care Units.

Attachment A
Clarifications for Community Living Center (CLC) Life Safety Surveys: (Rev: 6-9-23)

photos below show casework in accordance with the VA definition. Casework does not require sprinkler protection, as discussed above.

Examples of casework (wardrobes/lockers) for which sprinklers are not required



Attachment A
Clarifications for Community Living Center (CLC) Life Safety Surveys: (Rev: 6-9-23)

Examples of casework (wardrobes/lockers) for which sprinklers are not required



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Clarifications for Community Living Center (CLC) Life Safety Surveys: (Rev: 6-9-23)

Examples of casework (wardrobes/lockers) for which sprinklers are not required



Attachment A
Clarifications for Community Living Center (CLC) Life Safety Surveys: (Rev: 6-9-23)

Appendix: Supporting Information

Item A. Issue 15 of the VHA-Joint Commission Clarification Document, effective date September 4, 2017

Issue 15. Sprinklers in small closets (other than clothes closets and wardrobes): Some hose cabinets, hose closets, and electrical closets that are shallow in depth have not been provided with sprinkler protection. If these cabinets and closets did not have doors, the space would be protected by the nearby building sprinkler(s). Because doors are present, some inspectors have wanted sprinklers to be installed within the cabinet or closet. In line with NFPA 13 (see below), The Joint Commission will permit sprinklers to be omitted from these spaces provided that the space is small enough so that a person cannot reasonably enter the space (see photo for example).

NFPA 13 (2016 edition):

8.1.1(7) Furniture, such as portable wardrobe units, cabinets, trophy cases, and similar features not intended for occupancy, does not require sprinklers to be installed in them. This type of feature shall be permitted to be attached to the finished structure.

Please note that the Healthcare Interpretations Task Force (12-2008) and the 2018 edition of the Life Safety Code (18/19.3.5.10) have addressed the requirements for sprinklers within clothes closets and wardrobes.

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Item B. Applicable Code References

NFPA 13, 2016 Edition:

8.1.1(7) Furniture, such as portable wardrobe units, cabinets, trophy cases, and similar features not intended for occupancy, does not require sprinklers to be installed in them. This type of feature shall be permitted to be attached to the finished structure.

A.8.15.8.2 Portable wardrobe units, such as those typically used in nursing homes and mounted to the wall, do not require sprinklers to be installed in them. Although the units are attached to the finished structure, this standard views those units as pieces of furniture rather than as a part of the structure; thus, sprinklers are not required.

8.15.9* Hospital Clothes Closets. Sprinklers shall not be required in clothes closets of patient sleeping rooms in hospitals where the area of the closet does not exceed 6 ft² (0.55 m²), provided the distance from the sprinkler in the patient sleeping room to the back wall of the closet does not exceed the maximum distance permitted by 8.5.3.2.

A.8.15.9 This exception is limited to hospitals as nursing homes, and many limited-care facilities can have more combustibles within the closets. The limited amount of clothing found in the small clothes closets in hospital patient rooms is typically far less than the amount of combustibles in casework cabinets that do not require sprinkler protection, such as nurse servers. In many hospitals, especially new hospitals, it is difficult to make a distinction between clothes closets and cabinet work. The exception is far more restrictive than similar exceptions for hotels and apartment buildings. NFPA 13 already permits the omission of sprinklers in wardrobes [see 8.1.1(7)]. It is not the intent of this paragraph to affect the wardrobe provisions of NFPA 13. It is the intent that the sprinkler protection in the room covers the closet as if there was no door on the closet (see 8.5.3.2).

NFPA 101, 2018 edition:

18/19.3.5.10* Sprinklers shall not be required in clothes closets of patient sleeping rooms in hospitals where the area of the closet does not exceed 6 ft² (0.55 m²), provided that the distance from the sprinkler

Attachment A
Clarifications for Community Living Center (CLC) Life Safety Surveys: (Rev: 6-9-23)

in the patient sleeping room to the back wall of the closet does not exceed the maximum distance permitted by NFPA 13.

A.18/19.3.5.10 This exception is limited to hospitals, as nursing homes and many limited care facilities might have more combustibles within the closets. The limited amount of clothing found in the small clothes closets in hospital patient rooms is typically far less than the amount of combustibles in casework cabinets that do not require sprinkler protection, such as nurse servers. In many hospitals, especially new hospitals, it is difficult to make a distinction between clothes closets and cabinet work. The exception is far more restrictive than similar exceptions for hotels and apartment buildings. NFPA 13 already permits the omission of sprinklers in wardrobes [see 8.1.1(7) of NFPA 13]. It is not the intent of 19.3.5.10 to affect the wardrobe provisions of NFPA 13. It is the intent that the sprinkler protection in the room covers the closet as if there were no door on the closet. (See 8.5.3.2.3 of NFPA 13.)

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Item C. Healthcare Interpretations Task Force Interpretation No. 2, December 2008

HITF INTERPRETATION
DECEMBER 2008 NO. 2

Document to be interpreted: NFPA 13 (1999) 5.13.9.2 and A5.13.9.2

Edition: 1999

Background Information (optional): **DETERMINE THAT RECESSED WARDROBES
IN AN ALCOVE ARE NOT “FREESTANDING”
WARDROBES**

We have a major AHJ who has determined that recessed wardrobes in an alcove are not “freestanding” Wardrobes per Sections 5.13.9.2 & A5.13.9.2 of the 1999 NFPA 13 and are required to have sprinklers installed inside the wardrobes. The nursing home industry has 1000’s of resident sleeping rooms with recessed wardrobes and to the best of my knowledge these wardrobes have never been cited for not having sprinklers inside the wardrobes. I suspect that many hospitals also have recessed wardrobes that do not have sprinklers inside the wardrobes. Please see the following attachments:

1. My letter to NFPA for a staff interpretation.
2. Letter from Jim Lake, NFPA staff liaison to NFPA 13 responding to my letter.
3. Pictures of a typical installation that has been cited for the lack of sprinklers.

Question: Do recessed wardrobes (attached or freestanding) in an alcove that is fully sheathed with sheetrock qualify as freestanding wardrobes per Sections 5.13.9.2 & A.5.13.9.2 of the 1999 edition of NFPA 13 and therefore are not required to be sprinklered?

Answer: Yes. Provided that the sprinkler protection of the room includes the floor space of the alcove.

HITF Interpretation of June 2014

B. Transmission of Fire Alarm Signals: NFPA 101.

This question is helping to clarify if a fire alarm signal is intended to be transmitted off site during the fire drill exercises required by NFPA 101. The HITF agreed that verification of the off-site alarm transmission is a function of the NFPA 72 type testing as part of the ITM package of requirements.

By a vote of 7 – 0, the Voting AHJ members agreed to the response as follows:

QUESTION 1:

Is it the intent of NFPA 101-2000 Edition Section 19.7.1.2, and NFPA 101-2012 Edition Section 19.7.1.4 to require healthcare occupancies to confirm the transmission of a fire alarm signal all the way to the local fire responding agency, even if the fire alarm system off-premises transmission equipment is connected to a third-party monitoring company?

ANSWER 1:

No. The drill required by NFPA 101: Sections 19.7.1.2 (2000 ed.) / 19.7.1.4 (2012 ed.) is intended to test the readiness of the staff – not to validate the transmission of the alarm. That test verification takes place as part of the NFPA 72 Inspection, Test and Maintenance (ITM) criteria.

QUESTION 2:

If the answer to question 1 is “No”, is it the intent of NFPA 101-2000 Edition Section 19.7.1.2, and NFPA 101-2012 Edition Section 19.7.1.4 to require healthcare occupancies to confirm the transmission of a fire alarm signal to the third-party monitoring company if they are so equipped?

ANSWER 2:

No.

HITF Interpretation of June 2009

HITF INTERPRETATION **JUNE 2009 NO. 1**

Document to be interpreted: NFPA 101, 2000 edition, 19.2.7.

Edition: 2000

Background Information (optional): **EXIT DISCHARGE**

Issue: Many existing health care facilities have exits that discharge to surfaces in accordance with the provisions of Chapter 19.2.7 and 7.7.1 such as grass lawns, or dirt and gravel yards. Authorities Having Jurisdiction are now determining these exits to be deficient and are requiring that all existing exits discharge to a paved hard surface sidewalk to a public way.

Some of these existing health care facilities are rural and located far distances from a public way so that it would be impracticable to extend the exit discharge all the way “to a public way”.

Background Information:

In Reference to Question 1, Exit Discharge Obstructions:

19.2.7 Discharge from Exits. Discharge from exits shall be arranged in accordance with Section 7.7.

7.7.1 Exits shall terminate directly at a public way or at an exterior exit discharge. Yards, courts, open spaces, or other portions of the exit discharge shall be of required width and size to provide all occupants with a safe access to a public way.*

A.7.7.1 Exterior walking surfaces within the exit discharge are not required to be paved and often are provided by grass or similar surfaces. Where discharging exits into yards, across lawns, or onto similar surfaces, in addition to providing the required width to allow all occupants safe access to a public way, such access also is required to meet the following:

- (1) The provisions of 7.1.7 with respect to changes in elevation*
- (2) The provisions of 7.2.2 for stairs, as applicable*
- (3) The provisions of 7.2.5 for ramps, as applicable*
- (4) The provisions of 7.1.10 with respect to maintaining the means of egress free of obstructions that would prevent its use, such as snow and the need for its removal in some climates.*

7.1.10 Means of Egress Reliability.

7.1.10.1* Means of egress shall be continuously maintained free of all obstructions or impediments to full instant use in the case of fire or other emergency.

Attachment C
Clarifications for Community Living Center (CLC) Life Safety Surveys: (Rev: 6-9-23)

A.7.1.10.1 A proper means of egress allows unobstructed travel at all times. Any type of barrier including, but not limited to, the accumulations of snow and ice in those climates subject to such accumulations is an impediment to free movement in the means of egress.

This section of the LSC has changed very little from the 1985 edition except that the language now incorporates a formal interpretation from that edition. The formal interpretation of the 1985 edition reads as follows:

Chapter 5 of 1985 LSC: FI 81-33, Reference 5-7.1.

Question 1: In a health care occupancy, are sidewalks required between the exit door and the public way in order to qualify as an exit discharge as stated in Paragraph 5-7.1?

Answer: No.

Question:2: If the answer to Question 1 is “no”, then is an open and unobstructed yard large enough to provide all occupants with a safe access to a public way acceptable as an exit discharge?

Answer: Yes, however, the path of safe access to the public way must also meet 5-1.6 with respect to changes in elevations and 5-1.7.3 with respect to maintaining the means of egress free of obstructions which would prevent its use, such as snow in some climates and the need for its removal.

There is nothing in this FI regarding grass, gravel, or dirt not being acceptable. In fact the answer to the first question is very clear that a sidewalk is not required and yet many AHJs are using paragraph 7.1.10 to not accept a discharge of grass, dirt or gravel because they assert that when it rains, these surfaces become wet and therefore may become an obstruction to egress.

It is easy to understand how snow or ice may become a barrier to the exit discharge and must be removed, but A.7.7.1 clearly indicates many “...walking surfaces within the exit discharge are not required to be paved and often are provided by grass or similar surfaces”. Still, because there is the presence of a grass lawn, the AHJs assert a wheelchair cannot be pushed through a grass lawn and require a paved surface from every exit discharge. The paving of these existing exit discharges is very costly and provides no measurable additional benefit to the safety of the patients or residents if they are properly maintained.

In Reference to Question 2, Exit Discharge Extension:

Although there is no definitive distance an exit discharge must extend when there is no public way in proximity to the building, there are some paragraphs in the Annex material of Chapter 7 that may give some indication how far it should extend.

A.7.8.1.1

Illumination provided outside the building should be to either a public way or a distance away from the building that is considered safe, whichever is closest to the building being evacuated.

A.7.9.1.1

Emergency lighting provided outside the building should be to either a public way or a distance away from the building that is considered safe, whichever is closest to the building being evacuated.

These paragraphs provide some additional insight into the thinking of the Technical Committee on Means of Egress. That is, the primary purpose of exterior egress route is for the occupants to

Attachment C
Clarifications for Community Living Center (CLC) Life Safety Surveys: (Rev: 6-9-23)

be able to move to a public way or a distance *considered safe* from the building, whichever is closest.

Often some AHJs use 50 ft. as a safe distance away from the building. The 50 ft. distance comes from Sections 22 & 23.2.7.1 for detention and correctional facilities. Others use 30 feet and still others 40 feet. How far does the Interpretations Task Force think a person must move away from the building to be at a distance “considered safe”.

Questions:

Question 1:

In a healthcare occupancy, are sidewalks required between the exit door and the public way in order to qualify as an exit discharge as stated in Section 7.7.1?

Answer 1: NO.

See NFPA 101: Annex Section A.7.7.1.

Question 2:

If the answer to Question 1 is NO, when is “grass or similar surfaces” not acceptable as a means of exit discharge?

Answer 2:

See NFPA 101: Annex Section A.7.7.1 that provides guidance on the appropriate conditions to utilize a non-paved surface.